

Immunotherapies

Immunotherapies use stimulation of the body's own immune system to clear infected lesions.

Treatment Options for Anogenital Warts (AGW)

Ablative Techniques

- **Cryotherapy**
- **Mode of action:** Liquid nitrogen freezes and destroys lesions
- **Schedule:** Applied directly to lesions; typically repeated every 1–2 weeks until clearance
- **Clearance rate:** Up to 93%
- **Recurrence rate:** Varies, generally 20–30%
- **Advantages:** Non-invasive, no need for anesthesia, can be performed in outpatient settings
- **Disadvantages:** May cause blistering, pain, or pigmentation changes; multiple visits required
- **Surgery**
- **Mode of action:** Surgical removal using scissors or scalpel
- **Schedule:** Single procedure under local or general anesthesia
- **Clearance rate:** Up to 93%
- **Recurrence rate:** Moderate
- **Advantages:** Effective for large or obstructive lesions; immediate removal
- **Disadvantages:** Requires anesthesia; risk of post-operative pain, scarring, and longer recovery time
- **Trichloroacetic Acid (TCA; 33–50%)**
- **Mode of action:** Causes chemical destruction of warts

- **Schedule:** Applied by physician up to three times per week until clearance
- **Clearance rate:** 70–100%
- **Recurrence rate:** High
- **Advantages:** Easy to apply, effective for small and targeted lesions
- **Disadvantages:** Common side effects include local discomfort, burning, and ulceration; requires skilled application; not preferred due to recurrence and adverse effects

Frequently Asked Questions: Patient Guidance

- **How did I get AGW?**

Anogenital warts (AGW) are caused by human papillomavirus (HPV). The virus is usually transmitted through sexual contact; indirect transmission is rare.

- **What is the risk of HPV transmission?**

The risk is very high—on average, only 1.6 sexual encounters are needed for transmission. HPV infection is extremely common, and most people will be infected at some point in their lives.

- **Is there a treatment?**

Yes, several treatment options exist, but they remove visible warts and do not eliminate the virus. Recurrence is possible even after successful treatment.

- **Does smoking increase my risk of developing AGW?**

Yes, smokers are at higher risk. Smoking cessation is strongly encouraged to reduce the risk of developing and recurring warts.

- **How long will I have AGW?**

Warts may recur, but with appropriate treatment, most cases resolve within 3 months.

- **Is this the end of my sex life?**

No. Patients can maintain a normal sex life. Open communication with partners and safe practices are important.

- **Should I disclose to my current and previous partners?**

It is important to inform your current partner so they can be examined and treated if necessary. Disclosure to previous partners may be considered, though transmission timing is often unclear.

- **Should I always use a condom?**

Increased condom use is associated with higher rates of HPV clearance. Routine condom use is recommended to reduce transmission risk.

- **What are the risks during pregnancy?**

AGW may grow larger during pregnancy due to hormonal changes but typically resolve within weeks after delivery. Rarely, HPV can be transmitted during childbirth, potentially causing recurrent respiratory papillomatosis in the infant.

- **Will I develop cancer?**

AGW themselves are not cancerous. They are caused by low-risk HPV types (e.g., 6 and 11). Other high-risk HPV types (e.g., 16 and 18) are associated with cervical and other anogenital cancers.

- **Can AGW spread to other parts of the body?**

It is very uncommon for AGW to spread to other body sites. Autoinoculation (spreading to nearby skin) is possible but rare.

Abbreviations: AGW, anogenital warts; HPV, human papillomavirus